DEPARTMENT OF HEALTH &. HUMAN SERVICES
Public Health Service
Indian Health Service
Phoenix Indian Medical Center
4212 North 16th Street
Phoenix, Arizona 85016

Memorandum

To: Governing Body
Phoenix Indian Medical Center (PIMC)
Cc: Health and Human Services Office of Inspector General
From: Hon. Kevin D. Turnage, MSN Ed., RN, NHDP-BC, NPD-BC, AMB-BC, NE-BC
Date: May 21, 2025
Subject: Urgent Policy Noncompliance – Sentinel Event Framework Misalignment and Executive Inaction
Attachments: Appendix A: Documented Record of Communications and Disclosures
Appendix B: Policy Compliance Matrix

Executive Summary

This memorandum addresses critical compliance gaps in PIMC's current Safety Event Reporting Policy, specifically the absence of a clearly defined sentinel event framework. Despite multiple communications on May 9, 14, and 21, 2025, highlighting these issues and providing actionable recommendations, no corrective measures have been implemented. This oversight violates The Joint Commission (TJC) accreditation requirements, Centers for Medicare & Medicaid Services (CMS) Conditions of Participation (CoPs), and Indian Health Manual (IHM) standards, exposing the organization to increased survey vulnerability, patient safety risks, and reputational harm.

1. Summary of Prior Executive Communications

- **May 9, 2025:** Initial memorandum titled "Sentinel Event Memorandum for Record – Request for Review and Action" submitted.
- **May 14, 2025:** Chief Quality Officer (Mr. Clinton Pope) responded, asserting that existing policies sufficiently address sentinel events.
- **May 21, 2025:** Follow-up memorandum provided direct citations from TJC, CMS, and IHS guidelines, refuting previous claims and requesting policy revision or creation of a standalone sentinel event policy.
- **May 21, 2025:** Protected Disclosure filed under federal law (5 U.S.C. § 2302; 41 U.S.C. § 4712), citing gross mismanagement and systemic compliance failure, submitted with supporting documentation to Executive Leadership and Office of Quality.

2. Core Compliance Issues and Organizational Risk

A. Failure to Define "Sentinel Event" per TJC and IHS

- Current policy lacks a standalone definition.
- Contravenes IHM 3-42.1G(20): "Sentinel events are a type of adverse event defined as an unexpected occurrence involving death, serious physical or psychological injury, or risk thereof…"

B. Misclassification of Sentinel Events as General "Reportable Events"

- No clear escalation or tracking mechanism for sentinel events.
- No mandatory Root Cause Analysis (RCA) initiation timeline for high-harm incidents.
- Results in underreporting, survey noncompliance, and potential recurrence of preventable harm.

C. Absence of Required External Reporting Language

- Policy omits requirements under:
  - IHM 3-43.3C(5): "Sentinel events must be reported to appropriate external agencies…"
  - CMS Tag A-0309 and 42 CFR § 482.21(d)(2) requiring formal RCA and corrective action monitoring for adverse events.

D. Executive Admission of Nonconforming Policy Basis

- In email dated May 14, 2025, the Chief Quality Officer confirmed:

  "I shamelessly stole that term [reportable event] from DoD policy… I do not share your concerns in using this terminology."

- This admission demonstrates that the current policy was not developed based on CMS or Joint Commission regulatory frameworks.[1]

3. Operational Consequences of Ambiguous Terminology

- Frontline staff misclassify events, undermining RCA initiation and follow-through.
- Leadership is not reliably notified of high-severity incidents requiring immediate governance review.
- Patient suicides, surgical never events, or fatal medication errors may be logged and closed without the organizational learning required under TJC SE-3 to SE-6.
- Executive accountability is fragmented, and policy owners lack authoritative guidance to revise or align supporting procedures. [2]

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[1] Patra A, De Jesus O. Sentinel Event. In: StatPearls [Internet]. Treasure Island (FL): StatPearls Publishing; 2023. Available from: https://www.ncbi.nlm.nih.gov/books/NBK564388/. The review emphasizes that all healthcare organizations should maintain a policy for **responding to sentinel events**, including timely root cause analysis, system-level corrective actions, and disclosure to appropriate stakeholders, in accordance with Joint Commission expectations.

[2] AliMed. *Best Practices for Sentinel Events: What Every Healthcare Organization Needs to Know*. March 7, 2019. Available from: https://www.alimed.com/blogs/staff-and-patient-safety/best-practices-for-sentinel-events-what-every-healthcare-organization-needs-to-know. The article stresses the need for accredited organizations to define sentinel events, establish a response system including root cause analysis within 45 days, and foster a transparent reporting culture based on trust, feedback, and non-punitive reporting.

4. Summary of Affected Regulations and Standards

The following table outlines key regulatory standards and the corresponding gaps identified in PIMC's current policies:

Standard	Requirement	PIMC Gap
TJC LD.03.09.01	Leadership manages sentinel events	✕ Absent in policy
42 CFR § 482.21(d)(2)	Implement improvement actions after adverse events	✕ No sentinel-specific RCA
IHM 3-42.1G(20); 3-43.3C(5)	Define and report sentinel events externally	✕ Omitted
CMS SOM Tag A-0309	Require RCA for harm-level events	⚠ Not clearly mandated

5. Recommendations for Remedial Action

A. Mandate the Development of a Standalone Sentinel Event Policy

- Define sentinel events per IHS and TJC standards.
- Establish explicit RCA, executive notification, and reporting procedures.
- Integrate with existing I-STAR, Risk Management, and Medical Staff Bylaws policies.

B. Direct Immediate Revision of the Current Safety Event Reporting Policy

- Clarify the scope and limitations of the “reportable event” term.
- Include clear cross-references to sentinel event procedures and escalation protocols.[3]

C. Initiate Staff Training and Quality Council Oversight

- Ensure all clinical and administrative personnel are educated on sentinel event recognition and required follow-up.
- Require quarterly reporting to the Governing Body on sentinel event response compliance.

The current absence of a formally defined sentinel event framework and the reliance on generalized "reportable event" terminology at PIMC significantly increase the likelihood of employee misinterpretation, reporting inconsistencies, and missed identification of high-risk events. Without clear definitions, escalation protocols, and training aligned with national standards, staff are left to apply subjective judgment in classifying events, often without understanding which incidents meet criteria for immediate Root Cause Analysis (RCA) and leadership notification. This ambiguity compromises both regulatory compliance and patient safety.

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[3] Simbo AI. *The Evolution of Sentinel Event Reporting: How Continuous Improvement Enhances Transparency and Patient Care Quality*. Simbo AI Blogs. 2024. This article highlights that sentinel events require immediate investigation and structured response, underscoring the importance of a formal sentinel event policy to promote transparency, reduce risk, and implement sustainable quality improvements across healthcare environments.

Moreover, the policy's lack of alignment with established best practices—as outlined by The Joint Commission, CMS, and peer-reviewed guidance (e.g., NCBI StatPearls, AliMed, and Simbo AI)—places the organization out of step with current standards of practice. Sentinel events are universally recognized as requiring urgent and structured responses; failure to explicitly define and manage them undermines the organization's ability to prevent recurrence, erodes accountability, and threatens both accreditation status and public trust.

This issue has now been raised through multiple formal channels, including a whistleblower disclosure. The continued failure to act invites reputational, legal, and operational risk. It is the Governing Body's responsibility to ensure that policies reflect current federal standards, support accurate reporting, and ultimately safeguard patient safety.

Respectfully submitted,

Digitally signed by Kevin D. Turnage -S
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Appendix A: Documented Record of Communications and Disclosures

May 9, 2025
Subject: Submission of Sentinel Event Memorandum for Record – Request for Review and Action
Summary: Outlined the absence of a Sentinel Event Policy and requested immediate review. Identified 13 related policies affected.

May 9, 2025 (Response)
Subject: Chief Quality Officer Response
Summary: Admitted use of DoD language. Declined to distinguish sentinel events, said concerns were already addressed.

May 14, 2025
Subject: Follow-up from Turnage
Summary: Cited specific Joint Commission and CMS requirements. Reiterated policy deficiencies.

May 14, 2025 (Response)
Subject: Leadership Response
Summary: Maintained current policy is sufficient. CEO concurred with CQO's position.

May 21, 2025
Subject: Updated Memorandum Submission
Summary: Provided full regulatory context and proposed solutions. Reaffirmed need for policy revision.

May 21, 2025
Subject: Protected Disclosure Filed
Summary: Filed formal complaint under whistleblower protection, citing gross mismanagement and patient safety risk.

Appendix B: Policy Compliance Matrix

Policy Title	Policy ID (if applicable)	Compliance Gap Description
Healthcare Risk Management and Patient Safety	16495443	Sentinel event definition, however, has no RCA expectations or escalation pathways.
After Hours Hospital Oversight	14936122	Lacks integration with event escalation triggers for sentinel events.
Clinical Alarm Safety and Management	11284239	May involve patient harm; no tie-in to sentinel event classification.
Code Grey: Abusive or Violent Conduct	155589764	Does not define when patient harm from violence qualifies as sentinel.
Code Stork Response	17992512	No protocol for RCA if obstetric/neonatal events result in harm or death.
Medical Staff Bylaws	16766145	Mentions sentinel events but lacks cross-referenced policy structure.
Medication Events	14059333	Fails to differentiate harm-level medication events as sentinel.
Point of Care Testing Quality Management	17697960	No escalation framework if POCT leads to adverse outcome.
Safety Event Reporting	16969672	Does not separately define or manage sentinel events.
Sentinel Events – Departmental – Lab	13676963	May contain outdated or departmental-only guidance; not aligned facility-wide.
Compliance Plan	17020676	Omits oversight language for sentinel events under compliance or audit.
Medical Device Failure and Recall Policy	14919814 14919361	Device-related deaths/injuries are not flagged for sentinel event tracking or RCA.
General Risk Management Policy	N/A	Overarching risk policies do not include sentinel-specific triggers or reporting duties.